Clinical Findings of Herpes Zoster

Prodrome of Herpes Zoster

Pain and paresthesia in the involved dermatome often precede the rash by several days. These sensations can range from mild itching, tingling, or burning to severe, deep, boring, or lancinating pain. The pain may be constant or intermittent and is frequently accompanied by tenderness and hyperesthesia in the affected dermatome.

Pre-eruptive pain can mimic a variety of conditions such as pleurisy, myocardial infarction, duodenal ulcer, cholecystitis, biliary or renal colic, appendicitis, prolapsed intervertebral disk, or early glaucoma. This may lead to serious misdiagnosis and inappropriate interventions. Prodromal pain is uncommon in immunocompetent individuals under 30 years of age but occurs in most patients over 60.

In rare cases, patients experience acute segmental neuralgia without developing a skin rash—a condition referred to as **zoster sine herpete**.

Rash of Herpes Zoster

The most distinctive feature of herpes zoster is the unilateral distribution of the rash, typically confined to the skin area innervated by a single sensory ganglion. The rash most commonly affects the dermatomes supplied by the **trigeminal nerve** (especially the ophthalmic division) and the thoracic segments **T3 to L2**. The thoracic region alone accounts for over half of all cases, and lesions rarely extend beyond the elbows or knees.

Although individual lesions of herpes zoster and varicella are histologically identical, those in herpes zoster evolve more slowly. They usually appear as **closely grouped vesicles on an erythematous base**, in contrast to the more scattered, discrete vesicles seen in varicella. This clustering reflects

intraneural spread of the virus in herpes zoster, as opposed to **hematogenous (viremic) spread** in varicella.

Lesions begin as **erythematous macules and papules**, often appearing first at sites where superficial branches of the affected nerve emerge—such as the posterior and lateral branches of spinal nerves. Vesicles develop within **12 to 24 hours**, progress to **pustules by day 3**, and then **dry and crust over within 7 to 10 days**. Crusts typically remain for **2 to 3 weeks**.

In immunocompetent individuals, new lesions appear for **1 to 4 days**, though occasionally up to **7 days**. The rash is most severe and prolonged in older adults and mildest and shortest in children.

In some cases, vesicles may appear outside the primary dermatome, indicating hematogenous dissemination, which is not uncommon.